

CompoundIQ POC Demo — Detailed Walkthrough

Version: 2.11 | **Date:** September 10, 2026 **Application:** <https://functional-medicine-infrastructure.vercel.app> **Duration:** 30–45 minutes (with discussion)

What's new in v2.11 (2026-09-10): A "Before You Present" checklist now sits at the top of this document, and the Ops catalog screen no longer contradicts the product-count claim. (a) New **Before You Present** section — five items to run in the five minutes before the audience joins: warm the routes (a cold serverless route measured ~**37 seconds** on first hit after a deploy vs **2-5 seconds** warm), confirm all four logins, create any live prescription during the demo because payment links expire, don't deploy on demo day, and keep the demo tab in the foreground (a backgrounded Chrome tab does not fire `requestAnimationFrame`, which stalls React's reveal of streamed content and makes a healthy page look hung). (b) **Part 5E rewritten** — `/ops/catalog` now leads with a read-only **Product Catalog** block showing live ingredient / salt-form / formulation / pharmacy-offering counts from the hierarchical catalog the prescription builder actually uses, and the CSV uploader below it is retitled "**Legacy Pharmacy Price List (CSV upload)**" with its own item count. Previously the page read "Catalog Management — 6 items," where the 6 were rows in the legacy flat price-list table — a prospect clicking Ops → Catalog after hearing "167 products" saw an apparent contradiction.

What's new in v2.10 (2026-09-09): The state-licensure guard is now a scripted feature beat, and the partial-load behavior it produces has been corrected in the product. (a) New **Part 3C-1** turns the per-state pharmacy licensure check into a headline selling point, run with **Jordan Rivera (CA)** against the Favorites tab, with the full pharmacy license matrix for reference. (b) New **safe-path warning box** at the top of Part 3: the scripted path uses **Alex Demo (TX)** because TX is the only state where all five pharmacies are licensed — improvising onto another patient can partially load or fully block a protocol, and the box names the specific traps. (c) **Partial protocol loads now advance.** Previously a protocol containing an item pinned to an unlicensed pharmacy loaded the licensed lines and then stranded the user on the quick-actions panel behind a red error. It now navigates to Review and carries a non-blocking **amber** notice naming each skipped medication and why; re-loading the same protocol no longer duplicates lines. (d) **Favorites and Protocols counts refreshed** — the Favorites tab reads **Favorites (10)** because `/api/favorites` is clinic-wide, and Protocols reads **Protocols (3)**. (e) **Post-#119 bundle wording** — the order drawer flips to the bundle panel in place the moment Combine succeeds; steps 37d/37e no longer tell the presenter to reopen the drawer. (f) **Table count corrected** from 33 to the verified **47 tables + 6 views** (PR #118 / GAP-3 added `protocol_template_versions`, `protocol_instances`, `order_clarifications` and three gate views).

What's new in v2.9 (2026-08-15): Demo data expansion: multi-provider, multi-state patients, lifecycle orders, second clinic (`scripts/demo-expansion-seed.sql`, run against prod). Sunrise now has 4 providers (Dr. Chen remains the only login/signer; Dr. Marcus Patel, Dr. Elena Rodriguez, and Jamie Fletcher NP add roster realism and make the F-3 clinic-view toggle + F-5 primary-provider features demonstrable), plus a second clinic — **Blue Cedar Integrative Health** (Dr. Naomi Osei, patient Ruby Sandoval, NM) — for the ops multi-tenant view. 8 new multi-state Sunrise patients (CA/NY/FL/WA/CO/AZ/IL/GA), 17 pharmacy state licenses (every patient state covered; CA has exactly 2 licensed pharmacies for the state-filter beat), and 12 lifecycle orders `DEMO-1001 .. DEMO-1012` across the pipeline (delivered/shipped/processing/failed/awaiting payment) on new-catalog medications, plus 6 provider favorites and a new "**Menopause Foundation — BHRT**" protocol by Dr. Rodriguez. See the new **Demo Cast & Story Beats**

subsection below the seed-data table, and the three new narration beats it introduces. Part 3B updated: the provider is now a real selection (4 providers), not auto-selected.

What's new in v2.8 (2026-08-15): Four updates reflecting the post-catalog-reseed fixes verified live in the R10 walkthrough (PRs #109/#110/#111). (a) **Protocol templates now price live** — loading a protocol pulls each medication's real wholesale price and applies the clinic's default markup, and the Mold/MCAS Support protocol now includes **Ketotifen Capsule 1mg** (see the live-pricing note in Part 3C). (b) **Bundle-link recovery** — after Combine and Send, the order drawer of any bundled order shows a "**Part of a Payment Bundle**" panel with a **Copy Bundle Payment Link** button, so the link is recoverable at any time rather than only from the one-time toast (new step 37e). (c) **Anti-double-pay messaging** — a patient opening an old solo payment link for an order that has since been bundled sees a specific "part of a combined payment bundle" message instead of a payable checkout (new step 37f). (d) **Review & Send affordance** — the send button shows the hint "Sign in the signature box above to enable sending" until the provider draws a signature (step 35).

What's new in v2.7 (2026-07-07): Corrected retail-default vs 2x note and Semaglutide formulation count per live R9 walkthrough.

What's new in v2.6 (2026-07-07): Three substantive updates reflecting shipped changes. (a) **166-product catalog** — the POC seed now carries 166 compounded formulations across 79 ingredients and 13 therapeutic categories (replacing the prior five-medication seed), so the cascading builder and search steps name specific formulation cards. (b) **Phase C "Combine and Send"** — sibling prescriptions for the same patient + provider can be merged into a single bundled patient payment link (new sub-part in Part 3F), and the patient checkout renders them as one "Prescription Bundle · 2 prescriptions" instead of two separate links. (c) **Recomputed margin example** — the Semaglutide walkthrough now prices from a \$95 wholesale (\$95 → 2x → \$190 retail; \$14.25 platform fee; \$80.75 clinic net margin), replacing the prior \$150→\$300 example.

What's new in v2.5 (2026-04-27): Two minor doc-only updates to match the live app's current state. (a) The Ops detail drawer's SLA tab is rendered with the proper acronym capitalization ("SLA", not "Sla"); the script in Part 5A now matches. (b) The Ops pipeline now seeds demo orders across all four routing tiers (T1 API / T2 Portal / T3 Hybrid / T4 Fax), so the Tier-icon narration in Part 5A reflects the cross-tier diversity the presenter can actually click into. No flow changes; the live-demo script principle from v2.4 still holds.

What's new in v2.4 (2026-04-23): This walkthrough is now a **live-demo script** — the presenter uses the app the way a real user would, and the investor sees whatever the app genuinely produces as a result of those actions. Prior versions asked the presenter to pre-stage specific visual states via a backstage `/ops/demo-tools` page right before the demo started; that pre-staging is gone. The demo no longer depends on clicking refresh buttons, setting timestamps, or forcing any screen into a specific colour state. Where earlier revisions predicted exact card counts, exact colours, or exact timestamp values on the Ops screens, those predictions have been replaced with descriptions of what each screen is — the specific state at demo time reflects the real activity the presenter creates during the walkthrough.

Before You Present

Run this in the five minutes before the audience joins. Every time. It is short, it is boring, and it is the difference between a demo that feels instant and a demo that opens with a 37-second spinner.

1. Warm the routes (5 minutes before, non-negotiable)

The app runs on Vercel serverless functions. A route that hasn't been hit recently — or that hasn't been hit *at all* since the last deploy — pays a **cold start** on its first request. Measured in production on 2026-09-09/10: the first request to an ops route after a deploy took **~37 seconds**. Once warm, the same routes returned in **2-5 seconds** (/ops/pipeline fastest, /ops/adapters 4.7s, /ops/fax 3.7s, /ops/catalog 2.0s).

Visit each of these once, in a tab you then leave open:

- ☐ The clinic dashboard (/dashboard)
- ☐ /new-prescription
- ☐ /ops/pipeline
- ☐ /ops/sla
- ☐ /ops/adapters
- ☐ /ops/fax
- ☐ /ops/catalog

If a page does lag mid-demo, say something true and move on: *"That's a serverless cold start — the function hadn't been called yet, so the platform is spinning one up. It's a first-hit cost, not a per-request cost."* Do not freeze, and do not apologize for it as though it were a bug. It isn't one.

2. Confirm all four logins work

Before anyone joins, log in and out once as each of the four roles in the **POC Test Accounts** table below (Ops Admin, Clinic Admin, Provider, Medical Assistant). Use the credentials exactly as listed in that table — do not invent new ones. This doubles as route warming for the clinic app.

3. Payment links expire — create the live prescription *during* the demo

Patient payment links are valid for 72 hours, but demo state is refreshed by cron and a link minted hours early can be stale by the time you get to it. **Create the live prescription in Part 3 while the audience is watching**, then copy its link in Part 3F/3G. Do not pre-create it the night before and expect the "Copy Payment Link" beat to work.

4. Do not deploy on demo day

A deploy resets **every** route to cold and re-triggers the 30-second-plus first hit — including on routes you warmed ten minutes earlier. Freeze merges to `main` until the demo is over.

5. Keep the demo tab in the foreground

A backgrounded Chrome tab does not fire `requestAnimationFrame`. React uses it to reveal streamed content, so a page loading in a background tab can sit on its loading spinner indefinitely and then paint instantly the moment you switch back to it — the page was healthy the whole time. Keep the tab you are demoing in the foreground, and do not "pre-load the next screen in a background tab" while you talk. (This cost a full QA round to re-diagnose. One line here so nobody does it twice.)

Pre-Demo Setup

Three one-time items before demo day — no backstage pages, no state-staging, nothing to click "10 minutes before the demo."

1. **Authenticator app on your phone.** The EPCS-controlled-substance signing step in Part 3F uses standard TOTP two-factor authentication per DEA 21 CFR 1311. The demo provider (Dr. Chen) has a TOTP secret already configured server-side. You need the same secret loaded into a TOTP app on your phone so you can enter the rolling 6-digit code during the signing step. Any TOTP app works (Google Authenticator, 1Password, Authy, Bitwarden). See the **Authenticator Setup** subsection below for the exact secret values.
2. **Open browser tabs.** Prepare 3 tabs so you can switch roles quickly during the demo — one for the clinic app, one for the ops dashboard, one for the patient checkout link.
3. **Stripe test card ready.** 4242 4242 4242 4242 | Exp: 12/28 | CVC: 123 | ZIP: 78701 . You'll type this into the real checkout page in Part 4B.

Optional: a phone in your other hand to show the checkout page on a real mobile browser.

Authenticator Setup (one-time, before demo day)

Dr. Sarah Chen's EPCS TOTP secret is pre-enrolled server-side so the signing flow skips first-time QR enrollment. To enter the rolling 6-digit code during the demo, load the same secret into a TOTP app on your phone once:

Field	Value
Account label	CompoundIQ Demo – Sarah Chen
Issuer	CompoundIQ POC
Secret (Base32)	KBWXXKJSXT4XFKUGUZKI20YCNNYPUCVBH
Algorithm	SHA-1 (default)
Digits	6 (default)
Period	30 seconds (default)
otpauth:// URI	otpauth://totp/CompoundIQ%20Demo%20-%20Sarah%20Chen?secret=KBWXXKJSXT4XFKUGUZKI20YCNNYPUCVBH&issuer=CompoundIQ%20POC

Most authenticator apps accept either a QR scan of the `otpauth://` URI (generate one from a QR generator of your choice) or manual entry of the Base32 secret. Two devices recommended — a primary phone + a desktop TOTP client (e.g. 1Password, Bitwarden) — so a phone battery surprise doesn't block the demo. Verify by comparing codes across apps before the demo starts.

Clock drift note: TOTP codes are clock-sensitive. If your phone's clock drifts more than ~30 seconds from the server, codes will be rejected. Make sure the phone's clock is set to automatic network time.

POC Test Accounts

Role	Email	Password	Redirects To
Ops Admin	ops@compoundiq-poc.com	POCAdmin2026!	/ops/pipeline
Clinic Admin	admin@sunrise-clinic.com	POCClinic2026!	/dashboard

Provider	dr.chen@sunrise-clinic.com	POCProvider2026!	/dashboard
Medical Assistant	ma@sunrise-clinic.com	POCMA2026!	/dashboard

POC Seed Data (reference — what the app is pre-configured with)

Entity	Details
Clinics	2 — Sunrise Functional Medicine (the demo clinic) + Blue Cedar Integrative Health (second tenant for cross-clinic ops realism)
Providers	5 across 2 clinics — Sarah Chen (NPI 1234567890, TX — the only auth login + signer), Marcus Patel, Elena Rodriguez, Jamie Fletcher NP (all Sunrise), Naomi Osei (Blue Cedar)
Patients	10 — Alex Demo (TX, DOB 1985-06-15, SMS opt-in — the live-checkout patient) + 8 multi-state Sunrise patients (CA/NY/FL/WA/CO/AZ/IL/GA) + Ruby Sandoval (NM, Blue Cedar)
Pharmacies	5 configured across all 4 tiers — Strive (Tier 4 Fax), Quick Rx + Express Digital Rx (Tier 1 API), Portal Plus (Tier 2 Portal), Hybrid Labs (Tier 3 Hybrid) — with 17 state licenses covering every patient state (CA deliberately has exactly 2 licensed pharmacies; TX has all 5)
Medications	166 compounded formulations across 79 ingredients and 13 therapeutic categories — Women's Health/BHRT, Men's Health, Thyroid, Peptides, Weight Management (GLP-1), Sexual Health, Dermatology, Hair Restoration, LDN, IV Therapy, Longevity, Adrenal, Mental Health.
Orders	12 lifecycle demo orders (DEMO-1001..DEMO-1012) spread over ~3 weeks — 4 Delivered, 2 Shipped (with tracking numbers), 2 Pharmacy Processing, 2 Paid Processing, 1 Submission Failed (the ops triage beat), 1 Awaiting Payment (Blue Cedar) — plus whatever live orders the presenter creates during walkthroughs

Demo Cast & Story Beats (v2.9)

The expanded seed gives the presenter named characters to point at. Every row below is real seeded data — pick these patients in the patient selector and the app behaves exactly as narrated.

Patients

Patient	State	Primary Provider	Story beat
Jordan Rivera	CA	Dr. Chen	State-license filter demo — pick Jordan and the pharmacy list filters down to the exactly 2 CA-licensed pharmacies (Strive + Quick Rx)
Maya Thompson	NY	Dr. Chen	East-coast realism; Progesterone order currently in Pharmacy Processing
Ethan Brooks	FL	Dr. Patel	TRT story — Testosterone Cypionate 200 with Dr. Patel (Schedule 3 → Tier 4 fax routing)

Sofia Nguyen	WA	Dr. Rodriguez	BHRT story — Estradiol cream delivered, DHEA in flight; pairs with the "Menopause Foundation — BHRT" protocol
Liam Carter	CO	Dr. Chen	NAD+/longevity story — NAD+ Injectable shipped with tracking
Ava Martinez	AZ	Dr. Rodriguez	Biest 80/20 cream compounding at Strive
Noah Kim	IL	Dr. Patel	Tadalafil delivered + the one Submission Failed order (ops triage beat)
Grace O'Connor	GA	Dr. Rodriguez	LDN 4.5 maintenance, shipped
Ruby Sandoval	NM	Dr. Osei (Blue Cedar)	Second-clinic patient — her Awaiting Payment order proves cross-clinic isolation in the clinic app + cross-clinic visibility in ops
Alex Demo	TX	Dr. Chen	The original E2E checkout patient — unchanged; still the live-payment walkthrough subject

Providers

Provider	Clinic	Role in the demo
Dr. Sarah Chen	Sunrise	The only auth login + signer — every live signing flow goes through her
Dr. Marcus Patel	Sunrise	Roster realism + F-3 "My patients / All clinic orders" toggle + F-5 primary-provider (owns the TRT storyline)
Dr. Elena Rodriguez	Sunrise	Same, on the BHRT storyline; author of the "Menopause Foundation — BHRT" protocol
Jamie Fletcher NP	Sunrise	Mid-level realism — no seeded orders, showing a roster that isn't 1:1 with order volume
Dr. Naomi Osei	Blue Cedar	Second tenant — powers the ops multi-tenant view; Sunrise clinic users never see her data

Three new narration beats

1. **Patient-state pharmacy filtering.** Start a new prescription, pick **Jordan Rivera (CA)**, and narrate the pharmacy list: only the two CA-licensed pharmacies (Strive + Quick Rx) appear. Switch to a TX patient (Alex Demo) and all five come back. "The system will never let a clinic send a prescription to a pharmacy that isn't licensed in the patient's state — that filter is the license table, live."
2. **The provider toggle is now meaningful.** Log in as Dr. Chen and flip the F-3 **"My patients / All clinic orders"** toggle — with three prescribing providers at Sunrise the two views now genuinely differ (Chen's own patients vs. Patel's TRT and Rodriguez's BHRT orders appearing in the clinic-wide view).

3. **A living pipeline.** The dashboard and ops pipeline now show a real spread — orders in Paid Processing, Pharmacy Processing, Shipped (with tracking numbers), Delivered, plus exactly one **Submission Failed** order (order numbers DEMO-10xx). Use the failed order as the ops triage beat: filter to Errors, claim it, walk the History tab. "This is what Tuesday morning looks like — not an empty demo database."
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Part 1: The Problem (2 minutes — no demo, just talking)

"Independent functional medicine clinics prescribe compounding medications at scale but lack the infrastructure to do so profitably. They face three compounding problems:"

1. Sourcing is manual

"Finding which pharmacy is licensed in the patient's state, has the right formulation, at the best wholesale price — that's done with phone calls, spreadsheets, and tribal knowledge. There's no searchable marketplace for compounding pharmacy services."

2. Margin math is error-prone

"Clinics mark up compounded medications but calculate margins manually. No real-time pricing tools, no price locking, no platform fee transparency."

3. Fulfillment is fax-dependent

"The prescription gets faxed. Then you wait. No real-time tracking, no automated escalation when orders stall. Every order requires human intervention at multiple stages."

"CompoundIQ replaces this with an AI-native system that handles the full lifecycle: search, price, pay, route, track, and deliver. Let me show you."

Part 2: Login & Authentication (3 minutes)

Show the Login Page

1. Navigate to `https://functional-medicine-infrastructure.vercel.app/login`

2. **Point out:**

- Dark gradient background with CompoundIQ branding
- Clean, modern design (Geist font, Tailwind CSS design system)
- HIPAA badge indicating healthcare-grade security
- Feature bullet points with checkmarks

"This is the unified login for all three applications. The system uses role-based access control — where you go after login depends on who you are."

Demonstrate Role-Based Routing

3. Log in as **Clinic Admin:** `admin@sunrise-clinic.com` / `P0CCLinic2026!`

4. **Point out:** Redirected to `/dashboard` — the Clinic App

"Clinic users — admins, providers, medical assistants — all land on the clinic dashboard. They can only see their own clinic's data. Row-Level Security is enforced at the PostgreSQL level, not just application code."

5. Sign out

6. Log in as **Ops Admin**: ops@compoundiq-poc.com / POCAAdmin2026!

7. **Point out**: Redirected to /ops/pipeline — the Ops Dashboard (dark mode)

"Ops admins see a completely different application — the dark-mode operations dashboard. They have cross-clinic visibility for monitoring the entire order pipeline."

8. **RBAC Demo**: While logged in as ops, navigate to /dashboard

9. **Point out**: "Access Denied" page — ops cannot see the clinic app

"The access control works in both directions. Ops can't access clinic data through the clinic app, and clinic users can't access the ops dashboard. This is enforced at every level — middleware, RLS policies, and JWT claims."

10. Sign out

Part 3: The Clinic Workflow (15-20 minutes)

Stay on the safe path: Alex Demo (TX)

Everything from here to Part 4 is scripted against Alex Demo (TX), and that is deliberate. TX is the only state where all five pharmacies hold an active license (Strive, Quick Rx, Express Digital, Portal Plus, Hybrid Labs). On Alex, every favorite is clickable and every protocol quick-load loads clean, all lines, first time.

If you improvise onto a different patient, expect the licensure guard to fire. That is the product working correctly — but explain it rather than look surprised. The specific traps:

- **"Menopause Foundation — BHRT" + Jordan Rivera (CA) → partial load.** The Portal Plus line is not licensed in CA, so it is skipped and the rest load.
- **Any protocol + Maya Thompson (NY), Sofia Nguyen (WA), Grace O'Connor (GA), Liam Carter (CO), or Noah Kim (IL) → full block.** None of those states has a licensed pharmacy for every pinned item, and for these patients no item survives the check.

What a partial load actually does (shipped behavior): the licensed medications load into the session and the app **advances to the Review step exactly as a clean load does**. On Review you get a **non-blocking amber notice** at the top — "Loaded 2 of 3 medications from Menopause Foundation — BHRT" — listing each skipped medication and the reason, e.g. "Progesterone Capsule 100mg — Portal Plus Pharmacy is not licensed in CA." Amber, not red: nothing is blocked, the signature pad works, and you can sign and send the lines that did load. If you re-click the same protocol, it will not duplicate anything — it tells you the lines are already in the session.

What a full block does: nothing is added, so the app stays on the quick-actions panel with a **red** error naming every skipped item. There is nothing to review, so there is nothing to advance to. Recover by choosing a different protocol, or by switching to Alex Demo.

If a prospect asks you to try their own state, this is a great moment to lean in — see **Part 3C-1** for the scripted version of that beat.

3A — Dashboard Overview

1. Log in as **Clinic Admin**: admin@sunrise-clinic.com / POCCLinic2026!

2. **Point out the dashboard:**

- KPI cards at top (Total Orders, Revenue, Pending Payment, Completed)
- Order table with status badges (colored pills showing order state)

- Table/Kanban toggle in the top right

3. Click the **Kanban toggle** to show the board view

"The medical assistant starts their day here. They can see all orders at a glance, filter by status, and switch between table and kanban views."

4. **Point out the sidebar:**

- Navigation icons for Dashboard / New Prescription / Settings
- Active page highlighted
- Sign-out control at the bottom

"The sidebar gives quick access to the core clinic flows."

3B — Patient & Provider Selection (New in Phase 15)

6. Click "+ New Prescription" button

7. **Point out the patient/provider selector page:**

- Patient search with name filter
- Provider selector (Sunrise now has 4 providers as of v2.9 — pick **Sarah Chen**, the login/signing provider)
- "Continue to Pharmacy Search" button (disabled until both selected)

"The first thing the MA does is select the patient. The patient's shipping state auto-populates for all pharmacy searches — no manual entry. The provider is also selected upfront. Both stay pinned at the top of every screen throughout the flow."

8. Search for "**Alex**" — select **Alex Demo** (TX state badge visible)

9. Select provider **Sarah Chen** (the only provider with an auth login — Patel, Rodriguez, and Fletcher are seeded for roster realism and the F-3/F-5 features)

***Expect an amber hint on three of the four provider cards.** Dr. Patel, Dr. Rodriguez, and Jamie Fletcher NP each show "**No signature on file — will capture during review**" in amber underneath their name. That is correct and not an error: only Dr. Chen has a captured `signature_hash` on file, so her card is clean. The hint is telling the MA that if they pick one of the others, the signature will be drawn live at the review step rather than pulled from file. If a prospect asks, this is the honest answer: "we don't fake a signature we don't have — we tell you up front when one has to be captured."*

10. Click "**Continue to Pharmacy Search**"

3C — Quick Actions: Favorites + Protocols (New in Phase 18)

11. **Point out the session banner** at the top — Alex Demo + Sarah Chen pinned

12. **Point out the Quick Actions Panel** with two tabs: **Favorites** and **Protocols**

13. **Favorites tab** — the tab header reads **Favorites (10)**. The list is ordered by use count, most-used first:

1. Semaglutide 0.5mg weekly
2. Standard TRT — Cyp 200mg
3. TRT Cyp 200 — Weekly
4. Estradiol 0.1% Cream
5. LDN Starter — Titration
6. Biest 80/20

7. LDN 4.5 Maintenance
8. BPC-157 daily cycling
9. NAD+ Longevity
10. Tadalafil 10 Troche

Note the titration/cycling badges on the relevant favorites (LDN Starter, BPC-157).

"Provider favorites let you reorder common prescriptions in one click. No searching, no configuring — just click and go straight to pricing. Clicking a favorite lands directly on the margin page with the live wholesale price and the saved sig carried over."

Presenter line — say this before they ask: "You'll notice favorites here that Dr. Chen didn't save. Favorites are **clinic-wide**, not per-doctor — Dr. Patel's TRT setups and Dr. Rodriguez's BHRT setups show up in Dr. Chen's list too, ordered by how often the clinic actually uses them. That's intentional. When a new provider joins a practice, they inherit the practice's prescribing patterns on day one instead of rebuilding them from scratch." (A prospect **will** ask why another doctor's favorite is on this screen. Answer it first.)

14. Click **Protocols tab** — the tab header reads **Protocols (3): Weight Loss Protocol, Mold/MCAS Support, and Menopause Foundation — BHRT** (added in v2.9, authored by Dr. Rodriguez)
15. Click **Weight Loss Protocol** to expand — show the 3 medications with phase labels and sig text
16. **Point out** the "Load 3 Medications into Session" button

"Protocol templates are a market-first feature. One click adds an entire multi-medication protocol to the session. The provider reviews and adjusts per patient before signing."

Live pricing note (v2.8): loaded protocol medications price from the live catalog — each line pre-fills its real wholesale cost with the clinic's default markup applied, not a placeholder. The **Mold/MCAS Support** protocol is the crisp example: it loads **Ketotifen Capsule 1mg** (\$22 wholesale → \$30.80 retail at the clinic's 40% default markup), **Low Dose Naltrexone** (\$28 → \$39.20), and **Thymosin Alpha-1** (\$132 → \$184.80). If you expand a protocol during the demo, the prices you see are the real ones the margin builder will show.

3C-1 — The State-Licensure Guard (New in v2.10)

Run this beat. It is 90 seconds, it needs no setup, and it lands a compliance argument that no slide can. It is also the single most common objection-killer in this demo: every clinic owner in the room has either paid for this mistake or knows someone who has.

- 16a. Open the patient selector again ("+ **New Prescription**") and this time select **Jordan Rivera** — the **CA** state badge is visible on the card. Keep **Sarah Chen** as the provider and continue.
- 16b. Land on the Quick Actions Panel and stay on the **Favorites** tab. Same 10 favorites as before — but **3 of the 10 are now grayed out and un-clickable**, each carrying a red **"not licensed in CA"** pill.
- 16c. **Point at one of the grayed cards and read the line underneath it out loud.** It names the pharmacy explicitly — for example *"Portal Plus Pharmacy is not licensed in CA — choose a licensed pharmacy for this patient."* Read whatever the screen actually says; the three that gray out are the favorites pinned to a pharmacy with no active CA license.

"Watch what just happened. I didn't change a setting, I didn't run a report, I didn't ask anyone. I picked a patient who lives in California, and the platform immediately took three prescribing options off the table — and told me exactly why, by pharmacy name. The clinic **physically cannot** route this patient's prescription to a pharmacy that isn't licensed in her state. Not 'we'll warn you.' Not 'check the box to confirm.' The button is gone."

16d. **Land the value.** "This is the failure mode that costs real clinics real money. A pharmacy fills across a state line it isn't licensed in, and now you're looking at a board complaint, a refund, an insurance problem, and a very bad week. The usual defense is a spreadsheet somebody updates when they remember to. Ours is a license table checked at the moment of prescribing, on every single line, for every single patient."

16e. **Switch back to Alex Demo (TX)** and show the same Favorites tab with all 10 clickable. "Same clinic, same favorites, same provider. Texas patient — everything's open, because in Texas all five of our pharmacies are licensed. The guard isn't a blanket restriction; it's the actual license map, applied per patient."

16f. **If they push on protocols:** expand **Menopause Foundation — BHRT** while Jordan Rivera is selected. The unlicensed line is flagged in the expanded list with "**not licensed in CA — will be skipped**" before you commit. Click **Load** and the app loads the licensed medications, advances to Review, and shows the amber notice naming what was skipped and why. "It doesn't refuse to help me. It does the part it's allowed to do, hands me the rest of the visit, and puts the compliance problem in writing."

Pharmacy license matrix (reference — keep this in your back pocket):

Pharmacy	Tier	Licensed states
Strive	Tier 4 Fax	TX, CA, FL, AZ
Quick Rx	Tier 1 API	TX, CA, CO
Express Digital Rx	Tier 1 API	TX, NY, IL
Portal Plus	Tier 2 Portal	TX, NY, WA, GA
Hybrid Labs	Tier 3 Hybrid	TX, FL, CO, NM

Read down the TX column: every pharmacy. That is why the scripted path uses a Texas patient.
Read down CA: Strive and Quick Rx only — which is exactly why three favorites gray out for Jordan Rivera.

3D — Cascading Prescription Builder (New in Phase 17)

- Go back to Configure Prescription. Type "**Sema**" in the medication search — select the ingredient **Semaglutide**
- Point out** the cascading dropdown flow: ingredient **Semaglutide** → salt form **Semaglutide** (base — the builder auto-skips this level because it's the only salt option) → dosage form **Injectable Solution** → route **Subcutaneous** → formulation card. Selecting the single ingredient **Semaglutide** surfaces **5 formulations directly**: Injectable 2.5, 5, and 10 mg/mL, an Oral Capsule, and a Sublingual Tablet. (The two combination products, Semaglutide + B12 and Semaglutide + Niacinamide, are reached via the combination path by selecting one of their component ingredients, not from this single-ingredient formulation list.) The presenter picks the specific "**Semaglutide Injectable 5 mg/mL**" card. Its wholesale is **\$95**.

19. Select the **"Semaglutide Injectable 5 mg/mL"** card → **Point out the Structured Sig Builder:**

- Dose amount + unit + frequency dropdowns
- Timing dropdown (In the morning, At bedtime, etc.)
- Duration dropdown (For 30 days, Ongoing, etc.)
- Mode toggles: **Standard / Titration / Cycling**

20. Set dose: **10 units**, frequency: **Once weekly**, timing: **In the morning**

21. **Point out** the auto-generated sig: "Inject 10 units (0.10mL / 0.50mg) subcutaneous once weekly in the morning"

"The sig generates automatically with full unit conversion — mg, mL, and syringe units for injectables. No manual math. NCPDP-compliant with a 1,000-character limit counter."

22. **Show Titration mode** — click "Titration" toggle. Point out the amber panel with Start at / Increase by / Every / Up to fields

"For titration protocols like LDN, the provider sets start dose, increment, interval, and target. The sig generates: 'Take 0.1mL by mouth at bedtime. Titrate up by 0.1mL every 3-4 days as tolerated up to 0.5mL.' No competitor has this."

23. Click back to **Standard** mode. Select **Strive Pharmacy** in the Pharmacy & Pricing section

24. Select quantity, refills. **Point out the star button** next to "Continue — Set Retail Price"

"The star saves this configuration as a provider favorite for one-click reorder next time."

25. Click **"Continue — Set Retail Price"**

3E — Dynamic Margin Builder + Multi-Prescription

Set Retail Price note: The retail field pre-fills at **\$133.00** (the clinic's 40% Default Markup); the **\$190.00** figures below assume the presenter taps the **2x** button. Without that tap, the retail stays at the \$133.00 default and the platform fee, clinic margin, and \$286.00 bundle total are all smaller.

26. **Point out the Margin Builder** — Wholesale: \$95 (locked), retail price **pre-populated at \$133** (1.4x wholesale, from the clinic's 40% default markup), multiplier buttons, Sig field pre-filled

27. Click **2x multiplier** — retail updates to \$190, margin 50%, platform fee \$14.25 (15% of the \$95 spread), est. clinic margin \$80.75

"Full transparency. The retail price is pre-filled from the clinic's default markup setting (currently 40%), so the MA never types a number unless they want to override. Clicking 2x bumps it to a higher margin. The clinic sees exactly what they earn before committing. The sig is already pre-filled from the builder."

28. **Point out the three action buttons:**

- **"Add & Search Another"** — add this prescription and search for another medication
- **"Review & Send"** — go to batch review with all prescriptions
- **"Save as Draft — Provider Signs Later"** — save without signing (WO-77)

"The MA has three choices. They can add more prescriptions for the same patient, go straight to review, or save it as a draft for the provider to sign later."

29. Click **"Add & Search Another"**

30. **Point out** — back on configure page, session banner shows "**1 prescription in this session**"
31. Search for "**Testosterone**" → three results now appear (**Testosterone, Testosterone Cypionate, Testosterone Propionate** — topical/pellet testosterone is modelled as the bare "Testosterone", while the injectable esters are separate top-level ingredients). Select **Testosterone Cypionate** → **Point out DEA Schedule 3 warning banner**
32. Cascade: salt form **Cypionate** (auto-skips as the only option) → dosage form **Injectable Solution** → route **Intramuscular** → formulation "**Testosterone Cypionate Injectable 200 mg/mL**" → set dose + frequency
33. Select Strive Pharmacy, set retail price. The Testosterone Cypionate 200 mg/mL wholesale is **\$48**, and the same **2x multiplier** the demo uses puts its retail at **\$96.00**. Click "**Review & Send (2)**"

3F — Batch Review, Interaction Alerts & EPCS 2FA (New in Phase 19)

34. **Point out the batch review page:**
 - **Controlled Substance banner** at the top when any prescription in the session is DEA-scheduled (appears because Testosterone is Schedule 3)
 - **Drug Interaction Alerts section** — alerts are dynamic based on the medications in the current session. With Semaglutide + Testosterone (this walkthrough), an INFO-severity alert appears with clinical guidance. With different pairings (e.g. Ketotifen + Ketamine), a WARNING-severity alert appears instead. The alert text comes from the drug-interactions knowledge base.
 - Session banner showing the prescription count
 - One prescription card per medication with pharmacy, pricing, and sig
 - Combined totals (total retail, platform fee, total clinic payout)
 - "Remove" link on each card
 - "+ Add Another Prescription" button
 - Single provider signature pad

"The system automatically detects drug interactions and surfaces them with clinical guidance — severity-coloured: red for critical, amber for warning, blue for informational. DEA-scheduled compounds trigger the red banner and force the EPCS 2FA step before signing."

35. **Point out** that the send button starts disabled with the hint "**Sign in the signature box above to enable sending**" underneath it — then **sign** on the signature pad and watch the hint clear and the button enable

"No silent failures here. Until the provider actually draws a signature, the send button is disabled and says exactly why. The moment the signature lands, it lights up."

36. Click "**Sign & Send All 2 Prescriptions**" → Click "**Confirm & Send**" — watch the progress messages. (The EPCS 2FA modal surfaces here because Testosterone is Schedule 3 — enter the 6-digit code from your authenticator app.)
37. Navigate back to the dashboard to see the resulting orders — both should appear as "Awaiting Payment"

EPCS 2FA Demo Tip: The EPCS 2FA modal (6-digit TOTP input, DEA 21 CFR 1311 citation) triggers on the batch Review & Send flow whenever any DEA-scheduled compound is present in the current prescription session. Because Steps 31–36 already added Testosterone (Schedule 3) to the session, clicking "Sign & Send All Prescriptions" → "Confirm & Send" will surface the modal with the red

"EPCS Two-Factor Authentication Required" header, a Schedule badge, and "Verify & Sign" / "Cancel" buttons. Read the current 6-digit code from your authenticator app (the one you loaded via the Authenticator Setup subsection before demo day) and enter it.

"Two orders created from one signature, three SLA timers each. In the next step we'll merge them into a single patient payment link with Phase C **Combine and Send** — that's the primary flow for a multi-prescription visit. (A per-order payment link still exists for single-prescription orders.) The MA's workflow for a multi-medication visit: 45 seconds."

3F — Phase C: Combine and Send (New)

"Those two prescriptions we just signed are siblings — same patient (Alex Demo), same provider (Dr. Chen), both **Awaiting Payment**. Before Phase C, that meant two separate payment links the patient had to open and pay one at a time. Now we bundle them into a single checkout."

37a. On the dashboard, both Alex Demo orders (Semaglutide + Testosterone) show "**Awaiting Payment**." Click either one to open the **order drawer**. 37b. **Point out the "Combine into one payment link" picker** — it lists the sibling order (the other Awaiting-Payment Rx for the same patient + provider) with a selectable checkbox. 37c. Select the sibling order, then click "**Combine and Copy Payment Link**." 37d. **Watch the drawer flip in place — do not close it.** The moment Combine succeeds, the solo "**Copy Payment Link**" block disappears and is replaced, in the same open drawer, by the "**Part of a Payment Bundle**" panel showing the prescription count and bundle total ("**2 prescriptions · \$286.00**") with a "**Copy Bundle Payment Link**" button. The dashboard rows behind the drawer update immediately too. One bundled checkout link covering both prescriptions is already on your clipboard — Semaglutide \$190.00 + Testosterone Cypionate \$96.00; the **\$190.00** assumes the **2x** tap, it defaults to \$133.00. This is the link you'll paste in Part 4.

"One link, both prescriptions, one payment. The patient taps once and pays a single combined total instead of juggling two links. Notice the drawer rewrote itself the instant the bundle existed — there's no refresh, no reopen, no stale button sitting there offering to do something that's no longer valid. The per-order 'Copy Payment Link' button still exists for single-prescription orders — but when a patient has multiple prescriptions from one visit, Combine and Send is the default. This is the friction Phase C removed."

37e. **Bundle-link recovery (optional — narrate or demo).** The bundle panel is not a one-time state. Close the drawer, click **either** bundled order, and the same "**Part of a Payment Bundle**" panel and "**Copy Bundle Payment Link**" button are there — same link, same total.

"The bundle link isn't a one-shot copy. If the toast gets dismissed or the clipboard gets overwritten, open any bundled order's drawer and re-copy the same link — no re-bundling, no support ticket."

37f. **Anti-double-pay talking point** (no extra clicks needed — narrate, or demo it if you kept an old solo link): if the patient opens an *old* per-order payment link for an order that has since been bundled, the checkout refuses it with a specific message that the prescription is now **part of a combined payment bundle** — not a payable page, not a generic error.

"That's deliberate. Once prescriptions are bundled, there's exactly one way to pay — the bundle link. A stale solo link can never produce a second charge for the same prescription."

3F — Provider Signature Queue (Draft Flow)

"Now let me show you the draft flow — where the MA saves a prescription for the provider to sign later."

38. Click "+ **New Prescription**" again
39. Select **Alex Demo** + provider **Sarah Chen** → Continue
40. Search "**Sema**" → select **Semaglutide** → select **Strive Pharmacy**
41. Click **2x multiplier**, enter Sig: "**Draft flow demo**"
42. Click "**Save as Draft — Provider Signs Later**"
43. Navigate back to the dashboard — the new order appears with "**Draft**" status

"The MA saved this without the provider being present. No signature yet."

44. **Sign out** of clinic admin
45. **Log in as provider:** dr.chen@sunrise-clinic.com / POCProvider2026!
46. Click the "**Drafts**" tab on the dashboard

"The Drafts tab lives on the shared dashboard and is visible to both clinic_admin and provider roles — anyone in the clinic can see what's pending signature, not just providers. The provider just happens to be the one who can act on it." 47. Click on the draft order — **point out the amber "Awaiting Provider Signature" banner** 48. Click "**Review & Sign This Prescription**"

"The provider sees a dedicated sign page with all the details the MA entered — medication, pharmacy, pricing, directions. They just review and sign."

49. **Point out** the sign page: patient info, provider info, prescription details, financial summary, signature pad
50. **Sign** on the pad → Click "**Sign & Send Payment Link**" → Confirm
51. Verify redirect to dashboard — order now shows "**Awaiting Payment**"

"The MA prepared it, the provider signed it later. Different sessions, different logins, same result. This is how a real clinic works."

52. **Sign out**

3G — Get the Checkout URL (in-app, no terminal)

53. **Copy the patient checkout URL** — still logged in as the provider (or switch back to the clinic admin), on the clinic dashboard: a. Click any order showing "**Awaiting Payment**" to open the order drawer b. Click the emerald "**Copy Payment Link**" button — the checkout URL is now on your clipboard

*"In production, the patient gets this link in a text message when the order is signed. They tap it on their phone and land directly on checkout. In a live clinic we'd never copy-paste the link — we're doing that here only because this is a demo. If the link ever expires, the same button changes to **Regenerate Payment Link** and mints a fresh 72-hour URL in one click. (This per-order button is the single-order path; for a multi-prescription visit you'd instead use the combined link from Part 3F — Phase C, Combine and Send.)"*

Part 4: Patient Checkout (5-7 minutes)

4A — Checkout Page

1. Paste the **bundled checkout URL** copied in the Combine-and-Send step (Part 3F — Phase C) into a new tab (or into a mobile browser for extra impact)
2. **Point out:**
 - Clinic branding: "Sunrise Functional Medicine" displayed prominently

- **"Prescription Bundle · 2 prescriptions · \$286.00"** — the two sibling Rx now share one checkout instead of two separate links
- Two generic line items, each labeled "Prescription Service" — NOT the medication name
- A single **combined total of \$286.00** — the sum of the two retail prices set in Part 3E (Semaglutide \$190.00 + Testosterone Cypionate \$96.00; the **\$190.00** assumes the **2x** tap — it defaults to \$133.00). The patient pays this one combined amount, once.
- **Email field** ("Email for receipt") — required, above the Stripe payment form. Stripe auto-emails a branded receipt to this address when the charge succeeds.
- Stripe Elements payment form below (card + whichever wallet options the patient's device supports — e.g., Apple Pay in Safari on iOS, Google Pay in Chrome on Android, Cash App Pay, Bank, Affirm, Amazon Pay)
- Trust signals: "256-bit TLS Encryption", "Powered by Stripe"
- Footer: "Your payment info is encrypted and never stored by CompoundIQ"

"This is what the patient sees. No login, no app download, no account creation. They tapped a link in a text message and landed here — and both prescriptions from today's visit are on one page as a single bundle. Notice — no medication name anywhere. HIPAA compliance means zero Protected Health Information touches Stripe or appears on any patient-facing surface."

3. Point out the white-labeling:

"The patient sees their clinic's name and branding. They don't know CompoundIQ exists. This is a branded checkout experience for the clinic."

4B — Complete Payment

4. Enter email in the **"Email for receipt"** field: `test@example.com` (Stripe will email the branded receipt here)
5. Enter Stripe test card in the Stripe Elements form below: `4242 4242 4242 4242` | Exp: `12/28` | CVC: `123` | ZIP: `78701`
6. Click the **"Pay"** button — it shows the combined bundle total of **\$286.00** for the 2 prescriptions (Semaglutide \$190.00 + Testosterone Cypionate \$96.00; the **\$190.00** assumes the **2x** tap — it defaults to \$133.00)
7. Wait for the success page

4C — Success Page

8. Point out:

- Animated green checkmark (CSS-only draw animation)
- "Payment Received" heading
- The combined bundle amount (**\$286.00**) in green
- Order reference: `#213881c7` (first 8 chars of UUID, monospace font)
- "What Happens Next" card with 3-step progress:
 - Payment confirmed (check)
 - Prescription sent to pharmacy (pending)
 - Pharmacy will contact you — "Within 3–7 business days" (Tier 4 fax timing)

"The patient gets immediate confirmation with clear next-step expectations. The messaging adapts to the pharmacy's integration tier — API-connected pharmacies show '24–48 hours' because they have real-time status. Fax pharmacies show the longer timeline. No false promises."

9. **Point out:** No medication name on the success page either

"Still zero PHI. The patient knows their payment went through and roughly when to expect their medication. That's all they need."

4D — Expired Link Page (Optional)

10. Navigate to `https://functional-medicine-infrastructure.vercel.app/checkout/expired`

11. **Point out:**

- Clock icon, friendly message
- "Payment links expire after 72 hours for security"
- Instructions to contact clinic
- No order details revealed

"If a patient waits too long, they see this. No PHI exposed. The clinic can reissue a new order — the expired one stays as a permanent record."

Part 5: Ops Dashboard (7-10 minutes)

5A — Pipeline View

1. Open a new tab, navigate to `https://functional-medicine-infrastructure.vercel.app/login`
2. Log in as **Ops Admin**: `ops@compoundiq-poc.com` / `POCAdmin2026!`
3. **Point out the dark-mode dashboard:**
 - Pipeline stage groups in the left sidebar (Payment, Submission, Pharmacy, Shipping, Errors / Terminal)
 - Each stage has a count badge
 - Order table with columns: Order, Status, Clinic, Pharmacy / Tier, SLA, Assigned, Actions

"This is the operations nerve center. Every order across every clinic is visible here — as of v2.9 that genuinely means two clinics: Sunrise and Blue Cedar Integrative Health. The dark theme is intentional — ops teams monitor this all day, and dark mode reduces eye strain."

4. **Point out an order row:**

- Status badge (colored)
- SLA countdown or overdue indicator
- Tier icon (Tier 1 API / Tier 2 Portal / Tier 3 Hybrid / Tier 4 Fax — the demo seed has orders across all four tiers)
- Claim button

"Each order shows its SLA status. If an order is overdue, it shows in red with the exact hours overdue. Ops can claim orders to prevent duplicate work."

5. **Show the filter bar:**

- Filter by Clinic, Pharmacy, Tier, Date range
- Filters reset by selecting "All" in each dropdown

"Multi-dimensional filtering lets ops isolate specific issues — show me all Tier 4 fax orders from this week, or all orders from a specific clinic. With two clinics and the DEMO-10xx lifecycle orders seeded, the clinic filter and the Errors stage both have real content to show."

6. **Click an order** to open the detail drawer

7. Point out the tabs: Detail, History, Submissions, SLA

"Full drill-down into any order. The History tab shows every state transition with timestamps and who triggered it. The Submissions tab shows every adapter attempt. The SLA tab shows all deadline tracking."

5B — SLA Heatmap

8. Click "**SLA**" in the top nav

9. **Point out:**

- Filter pills (All Active, Breached, etc.)
- SLA breach cards with countdown timers
- Escalation tier indicators
- Acknowledge button

"The SLA engine runs every 5 minutes. When a deadline is breached, it appears here with a countdown to the next escalation. Three tiers: Slack alert, DM to ops lead, PagerDuty page. Acknowledging stops the escalation — it tells the system 'I'm on it.'"

5C — Adapter Health Monitor

10. Click "**Adapters**" in the top nav

11. **Point out what the page is:** a card for every configured pharmacy across all 4 integration tiers (Tier 1 API, Tier 2 Portal, Tier 3 Hybrid, Tier 4 Fax). Each card shows:

- A traffic-light health indicator (green / yellow / red for Degraded or Critical / slate for Idle)
- Circuit breaker state as a plain-English chip: **Online** (green) / **Degraded** (amber) / **Offline** (red bold). Chip is only shown when the pharmacy has circuit-breaker telemetry — a freshly-configured adapter with no traffic yet omits the chip entirely.
- 24-hour submission success rate + total submission count + failure count
- p50 / p95 / p99 latency percentiles
- A 24-hour submissions bar chart (green bars for successes, pink for failures)
- "Last success" relative timestamp
- Quick-action buttons (Disable Adapter, Force Tier 4)

"Every pharmacy integration is monitored in real time. The card's colour reflects what that pharmacy has actually been doing in the last 24 hours — 95%+ success with recent activity is green, degraded performance shifts to yellow, and a circuit-breaker-open state flips it red. A pharmacy we've configured but haven't routed traffic through yet shows **Idle** — neutral slate — instead of falsely flashing Critical. What you see on the grid right now reflects today's real submission activity."

Narrator cue: whatever card is coloured however is fine. The page's value is that it reports reality — green pharmacies are healthy, yellow pharmacies are ones ops should look at, red pharmacies need intervention, and the circuit breaker auto-cascades traffic to the next-highest tier when something's down. Narrate the colours you see in front of you and explain what each state means.

5D — Fax Triage Queue

13. Click "**Fax Queue**" in the top nav

14. **Point out what the page is:**

- Status filter pills across the top (All, Received, Matched, Unmatched, Processed, Archived)

- Queue metrics in the header (X new , Y unmatched , total count)
- A list of inbound fax rows — each one shows status, from-number, page count, relative received-at timestamp, matched pharmacy/order if any
- Click any row to open a triage detail panel on the right with the available actions

"Tier 4 pharmacies respond via fax. Inbound faxes land here with an OCR text preview. The system attempts to auto-match each inbound fax to an open order. When it succeeds, the fax moves to Matched. When it can't, the fax sits in Unmatched until a human decides what to do with it — the right-hand triage panel gives ops the tools to manually match the fax to an order or archive it. Tier 1 and 2 pharmacies skip this entire page because their responses come back via API, not fax."

Narrator cue: whatever rows you see (including none) is what the queue genuinely contains today. The page's value is that it gives ops a single place to resolve fax-borne pharmacy responses — narrate whatever's in front of you.

5E — Catalog Manager

16. Click "**Catalog**" in the top nav

17. **Point out — two catalogs, clearly separated on one screen (new in v2.11):**

- At the top, a read-only **Product Catalog** block with live counts: **Ingredients, Salt Forms, Formulations**, and **Pharmacy Offerings**. These are the hierarchical catalog the prescription builder cascaded through in Part 3D — the same numbers the builder is working from. (Counts are live, so read whatever the screen says.)
- Below it, "**Legacy Pharmacy Price List (CSV upload)**" with its own "**N price-list items**" count, the CSV drag-and-drop area, the Manual Entry form, and the tabs: Catalog, Versions, Normalized, API Sync.

"Two things live on this screen and they do different jobs. The top block is the product catalog — ingredients, salt forms, formulations, and which pharmacies offer each one at what price. That's the tree the prescription builder walked down when I picked Semaglutide a few minutes ago. The bottom half is the pharmacy price-list importer: a pharmacy sends us a flat CSV of what they stock and what it costs, we version it, and we flag any price that moves more than 10%. Every change is versioned, and the normalized view lets you compare the same medication across pharmacies."

Narrator cue: if a prospect asks why the price-list item count is small, the honest answer is the right one: "That's the raw CSV import table, not the product catalog — it only has the rows a pharmacy has actually sent us a price sheet for. The product catalog is the block above it."

Part 6: Architecture Highlights (3-5 minutes, no demo)

The 4-Tier Pharmacy Adapter

"The key innovation is the Pharmacy Adapter Layer. Instead of forcing every pharmacy onto one integration method, we meet them where they are:"

Tier	Method	Speed	Coverage
Tier 1	Direct REST API	Instant	~25% of pharmacies
Tier 2	Portal Automation (Playwright)	~5 min	~20%
Tier 3	Standardized API Spec	Instant	Future 30%+

Tier 4	Fax Fallback	~30 min	Universal
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"The routing is deterministic — always use the highest available tier. If it fails, cascade down. Fax is always the fallback. A single LifeFile API integration unlocks Empower, Belmar, UCP, and Strive — the largest pharmacy network in the country."

Security & Compliance

"HIPAA compliance is enforced at the infrastructure level, not just application code:"

- Row-Level Security on all 47 tables
- Per-state pharmacy licensure enforced at the point of prescribing — an unlicensed pharmacy cannot be selected, quick-loaded, or protocol-loaded for that patient (see Part 3C-1)
- Zero PHI in Stripe (metadata contains order_id only)
- Supabase Vault for all pharmacy credentials
- 30-minute session timeout with warning modal
- Supabase Realtime DISABLED (hard HIPAA requirement)
- All data at rest encrypted AES-256
- Phase C multi-Rx payment groups (live) bundle sibling prescriptions into one patient payment link; PHI redaction (Option B) on logs and non-clinical surfaces; role features F-3 (provider clinic-view toggle) and F-5 (primary provider)

Technology Stack

"Built on Next.js 16, Supabase (PostgreSQL 15+), Stripe Connect Express, Twilio, and DocumomFax. Deployed on Vercel serverless. 10 cron jobs handle SLA enforcement, payment expiry, adapter health, and daily ops digest. Everything is atomic — Compare-And-Swap patterns on every state transition prevent race conditions."

Part 7: Q&A / Wrap-Up

Key Metrics to Mention

Metric	Value
Order states	23-state machine with 47 valid transitions
SLA types	10 enforcement types with 3-tier escalation
Database tables	47 (PostgreSQL with full RLS) + 6 views
Cron jobs	10 Vercel cron jobs
Build phases completed	19 phases, 87 work orders (all merged; WO-87 formulation support in prod)
Phase C & roles	Multi-Rx payment groups (live), PHI redaction (Option B), provider clinic-view toggle (F-3), primary provider (F-5)
Hard constraints	16 non-negotiable rules
Test coverage	65 Playwright E2E tests (all browsers) + 87 jest unit tests. CI gates every merge.

Common Questions

Q: How do you handle controlled substances?

"DEA-scheduled compounds are explicitly excluded from the adapter layer. They're flagged at search time and forced to Tier 4 (manual fax) only."

Q: What about patient data privacy?

"Zero PHI touches Stripe. The checkout page shows 'Prescription Service' — never the medication name. SMS messages contain only the patient's first name and a URL. Row-Level Security ensures clinics can never see each other's data."

Q: How do you stop a clinic from sending a prescription to a pharmacy that isn't licensed in the patient's state?

"We don't warn them — we remove the option. Every pharmacy carries a state license table, and every prescribing surface checks it against the selected patient's shipping state: the pharmacy dropdown filters, favorites pinned to an unlicensed pharmacy gray out with the reason and the pharmacy name, and a protocol quick-load skips the unlicensed lines and tells the provider exactly which ones and why. There is no override checkbox. See it live in Part 3C-1."

Q: What's the revenue model?

"Per-transaction platform fee. The spread between wholesale and retail is split: clinic keeps their margin, platform captures 15% of the spread. Stripe processing fees come out of the platform's portion."

Q: How long to integrate a new pharmacy?

"Tier 4 (fax) works immediately — just add the fax number. Tier 1 (API) requires their REST endpoint and credentials. Tier 2 (portal) requires Playwright selectors for their web portal. Tier 3 is our published spec that pharmacies can adopt."

Post-Demo Checklist

- ☐ Answer all questions
- ☐ Share POC URL if appropriate
- ☐ Note any feedback or feature requests
- ☐ Schedule follow-up if interest